



Armadale Kalamunda Group

Third and Fourth Degree Tears Guideline

1. Purpose

To provide guidance for the diagnosis, management, treatment and follow up of obstetric anal sphincter injury and prevention of anal incontinence, defined as involuntary loss of flatus, faeces or urge incontinence.

The overall risk of obstetric anal sphincter injury is 1% of all vaginal births. About 3% of all Australian women who have a vaginal birth, and 5% of women having their first vaginal birth, will experience a third or fourth degree perineal tear. Increased awareness and training of detection, classification and consistent high standards of repair has led to the reduction of morbidity and litigation associated with anal sphincter repair².

To provide education on the four components of perineum protection to reduce the risk of third and fourth degree tear in women identified as at risk of third or fourth degree tear occurring.

2. Applicability

This guideline applies to the medical, midwifery and allied health disciplines of the Obstetrics and Gynaecology department of the Armadale Health Service.

3. Guidelines

A third-degree perineal tear is the partial or complete disruption of the anal sphincter muscles, which may involve the external sphincter muscle (EAS) and internal anal sphincter muscle (IAS). **Obstetric anal sphincter injuries (OASIS)** encompass both third- and fourth-degree perineal tears.

Classification of Perineal tears:

The Royal College of Obstetricians and Gynaecologists (RCOG) classifies perineal tears as follows²:

First degree tear: injury to perineal skin and/or vaginal mucosa

Second degree tear: injury to perineum involving perineal muscles but not involving the anal sphincter.

3a: less than 50% of EAS thickness.

3b: More than 50% of EAS thickness.

3c: Both EAS and IAS torn

4th Degree tear: is the disruption of anal sphincter with the disruption of the rectal mucosa.

If there is any doubt about the degree of third-degree tear, it is advisable to classify it to the higher degree rather than the lower degree.

If the tear involves only the rectal mucosa with an intact anal sphincter (buttonhole tear), it is by definition not a fourth degree tear. This is to be documented as such. If not identified a rectovaginal fistula may develop.

Anal incontinence is defined as the complaint of involuntary loss of flatus and/or faeces affecting quality of life.

3.1 Risk factors

- Birth weight >4kg
- Persistent occipito-posterior position
- Nulliparity
- Increased maternal age(>35 years)
- Asian Ethnicity
- Post-term birth
- Precipitate labour
- Previous OASIS
- Second stage longer than one hour
- Estimated HC>35cm
- Shoulder dystocia
- Instrumental births especially with forceps use
- Epidural pain relief, where risk may be indirectly associated with prolonged second stage of labour or instrumental delivery.
- Posterior fourchette to mid-anus<2.5cm
- Midline episiotomy

Episiotomy should always be done mediolateral (8 o'clock position as it is associated with fewer anal sphincter injuries. Aim for the episiotomy to have an angle of 60 degrees to the midline, should be 1-3cm from fourchette and at least 4cm in length. The evidence that episiotomy prevents OASIS and/or anal incontinence is conflicting. However, mediolateral episiotomy has been shown to reduce OASIS by 50% in Nullipara when conducting vacuum births.

Clinicians need to be aware of the risk factors for obstetric anal sphincter injuries (OASIS). Clinicians should be aware, however, that risk factors do not allow the accurate prediction of OASIS.

3.2 Reducing the risk during Pregnancy, labour and birth:

1. Counsel about mode of birth at:
 - 1.1. First visit
 - 1.2. Around 36 weeks
 - 1.3. Support the woman to make her own informed decision.
 - 1.4. Women should be advised to consider perineal self-massage or partner assisted after 34 weeks of pregnancy. This can help reduce the risk of perineal trauma including third and fourth degree perineal tears.
 - 1.5 Pelvic floor muscle training may help prepare for labour and birth and reduce the possibility of a third or fourth degree perineal tear.
2. During a vaginal birth:
 - 2.1. Communicate clearly, especially in final stages of second stage.
 - 2.2. Follow the woman's choices and confirm consent prior to intervention

- 2.3. Apply warm compresses during second stage of labour at the commencement of perineal stretching.
- 2.4. With a spontaneous vaginal birth, using gentle verbal guidance, to encourage a slow controlled birth of the fetal head and shoulders: support the perineum with the dominant hand holding the warm compress; apply counter-pressure on the fetal head with the non-dominant hand; if the shoulders do not deliver, apply gentle traction to release the anterior shoulder; and allow the posterior shoulder to be released following the curve of Carus.
- 2.5. When episiotomy is indicated, it should be performed: at crowning of the fetal head, using a medio-lateral incision, at a minimum 60 degree angle from the fourchette.
- 2.6. Offer an episiotomy if forceps or vacuum are required and this is woman's first birth. Vacuum is preferable to forceps delivery due to less severe perineal trauma with vacuum use. If instrumental delivery to be undertaken consider perineal support by second accoucheur to help prevent perineal trauma.
- 2.7. For all women, genito-anal examination following birth needs to be offered and, where informed consent is given be performed by an experienced clinician; include a rectal examination for all women, including those with an intact perineum.
- 2.8. All perineal trauma should be graded according to the Royal College of Obstetricians and Gynaecologists (RCOG) guideline and reviewed respectfully by a second, experienced clinician to confirm diagnosis and grading.
- 2.9. There is little high-quality evidence to inform optimal maternal position during second stage of labour to minimise perineal trauma. However, there is greatest incidence of intact perineum in all-fours and kneeling positions, lowest incidence of OASIS in standing and lateral positions. There is greatest incidence and degree of perineal trauma in sitting, squatting and birth-stool positions. Lithotomy and squatting position are associated with increased risk of OASIS.

3.3 Recognition and treatment:

- All women having had a vaginal birth with evidence of genital tract trauma will be examined systematically to assess the severity of trauma prior to suturing. If there is any suspicion of a third or fourth degree tear, inform the GP Obstetrician for further assessment and classification. The GP Obstetrician will inform the Specialist Obstetrician of a third or fourth degree tear. Classification of the tear is to be documented on the AKMR73.
- All women who have an operative vaginal birth will be examined by an experienced practitioner trained in the recognition and management of third and fourth degree perineal tears. Classification of the tear is to be documented on AKMR73.
- Suturing of the tear should take place as soon as possible after birth to reduce the pain experienced by the woman and reduce the risk of infection and bleeding occurring
NOTE – after a water birth a delay in suturing of one hour is advised.
- An indwelling catheter must be inserted for 24 hours as a painful perineum and regional anaesthesia may cause urinary retention.
- Repair of third and fourth degree tears is to be performed in an operating theatre (OT) under regional or general anaesthesia by a Specialist Obstetrician or by the GP Obstetrician under strict supervision by the on call Specialist Obstetrician. Informed written consent is required of the woman prior to repair.
- The use of broad spectrum antibiotics is recommended to prevent wound infection and dehiscence. Cefazolin 1gm or 2gms depending upon the weight of the woman should be given intravenously (IV) along with 500mg of Metronidazole IV in OT. Use Clindamycin

600mg IV if the woman is allergic to penicillin. This is followed by oral Augmentin Duo for 5 days or Clindamycin 300mg three times a day for 5 days if allergic to penicillin.

Medication	Route	Dose	Frequency	Duration
Cefazolin	IV	1 to 2 grams (dependent on weight)	Once in OT	
Metronidazole	IV	500mg	Once in OT	
Clindamycin (if allergic to penicillin)	IV	600mg	Once in OT	
Augmentin Duo	Oral	500mg	BD	5 days
Clindamycin (if allergic to penicillin)	Oral	300mg	TDS	5days

2.2.1. Suture material and method

- The extent of the injury can be fully observed and accurately graded in OT.
- Either monofilament sutures such as polydioxanone (PDS) or modern braided sutures such as polyglactin (vicryl) can be used with equivalent outcome. Use fine sutures such as 3-0 PDS and 2-0 vicryl to cause less irritation and discomfort.
- Figure of eight sutures should be avoided during the repair of OASIS because they are haemostatic in nature and may cause tissue ischaemia.
- Bury the surgical knots beneath the superficial perineal muscles to prevent knot migration to the skin.
- Where the torn internal anal sphincter (IAS) can be identified, it is advisable to repair this separately with interrupted or mattress sutures without any attempt to overlap the IAS.
- Overlap or end to end techniques especially in full thickness external anal sphincter tear is to be used for repair due to significant reduction in faecal incontinence and urgency 12 months post repair with this technique.
- For partial thickness (all 3a and some 3b) tears, an end-to-end technique should be used.
- In fourth degree tear, vicryl 3-0 continuous or interrupted suture not completely through rectal mucosa(submucosal technique) is suggested to minimise the risk of fistula formation.
- Ensure good anatomical alignment and consider cosmetic results as well.
- Rectal examination must be performed (with maternal consent) after completing the repair to ensure that the suture material has not accidentally gone through the rectal mucosa.
- Complete documentation of the repair method and suture materials used on AKMR72.1

2.2.2. Post-operative repair management

- Intermittent ice/cold pack therapy for the first 48-72hours for 10-20 minute intervals, to reduce perineal and vulval oedema.
- Indwelling catheter for minimum of 12 hours. Consider remaining for 24 hours if significant genital trauma or until swelling subsides.
- Regular analgesia to be prescribed on the medication chart. Avoid Analgesia with codeine. Avoid rectal suppositories in third & fourth degree tears.
- Oral antibiotic therapy for seven days.
- Prescribe regular laxatives and stool softeners for 7-10 days post repair to reduce incidence of wound dehiscence.
- Observe the wound daily to assess healing and provide the woman with an opportunity to express any concerns about the healing and discomfort. Provide education on perineal

hygiene, advising the woman to change her pad 2-3 hourly, wash her perineal and anal area after each void and bowel movement and to refrain from using powder or any topical steroid cream.

- Refer the woman to the physiotherapist for treatment as well as education about pelvic floor muscle exercises and continued follow up for 6-12 weeks as an outpatient.
- Women no longer need to remain in hospital until their bowels have been opened.
- Referral appointment with treating Specialist Obstetrician at 6-8 weeks to be made before discharge from the hospital.
- If a woman is experiencing faecal incontinence, referral to colorectal surgeon should be considered. Care consideration may include:
 - Endoanal ultrasound (EAU).
 - Anorectal manometry.
 - Consideration of secondary sphincter repair.
- Advise the woman of:
 - The outcomes of an anal sphincter injury,
 - Prognosis-that after EAS, 60-80% of women are asymptomatic at 12 months.
 - Signs of ongoing symptoms or consequences.
 - Resuming sexual activity.
 - Future management and the importance of follow up.
 - The effect of the injury on subsequent pregnancy management. When planning future births, it is recommended to seek advice on mode of delivery from Specialist Obstetrician

3.4 Next birth following third or fourth degree tears:

- All women who sustained OASIS in a previous pregnancy should be counselled about the mode of delivery and this should be clearly documented in the medical records.
- All women who have sustained OASIS in a previous pregnancy and who are symptomatic or have abnormal endo-anal ultrasonography and/or manometry should be counselled regarding the option of elective caesarean birth.
- There were no systematic reviews or randomised controlled trials to suggest the best method of delivery following OASIS. The risk of sustaining a further third- or fourth-degree tear after a subsequent delivery is 5–7%.
- The risks of a subsequent vaginal delivery after a thirddegree tear have been assessed, with 17% of women developing worsening faecal symptoms after a second vaginal delivery. This seemed to occur if there had been faecal incontinence beyond 3 months but resolution by 6 months after the index delivery.
- If after shared Decision Making, woman decides to have vaginal birth:
 - Women should be cared for by an experienced midwife where practical during their birth journey.
 - On call GP Obstetrician should be kept informed of the progress and advised to attend second stage of birth to work collaboratively to minimise trauma to the perineum.
 - During the second stage, encourage the woman to adopt a non-supine position and apply warm perineal compresses in second stage of labour.
 - At crowning and birth of the head the midwife should guard the perineum as written above and carefully control the birth of the head.

- Encourage the woman breath through contractions without forced pushing and to continue pushing gently during contractions so the midwife can control the birth.
- Deliver the posterior shoulder first or in conjunction with the anterior shoulder to remove pressure from the perineum.
- The role of prophylactic episiotomy in subsequent pregnancies is not known and therefore an episiotomy should only be performed if clinically indicated.

4. Legislative context

The following documents are required to give effect to this policy:

- [WA Health Clinical Handover Policy MP 0095](#)
- [EMHS Patient Identification and Procedure Matching Policy: 21](#)

5. Supporting information

The following documents support the implementation of this policy:

- [Spontaneous Labour and Birth Guideline](#)
- [Discharge Guideline – Obstetrics Procedure](#)

6. Compliance, monitoring and evaluation.

Compliance against this guideline will be evaluated through the completion of an audit against the Australian Commission on Safety and Quality in Health Care – Third and Fourth Degree Perineal Tears Clinical Care Standards.

7. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 6 – Action 6.5: The health service organisation:
 - a. Defines approved identifiers for patients according to best-practice guidelines
 - b. Requires at least three approved identifiers on registration and admission; when care, medication, therapy and other services are provided; and when clinical handover, transfer or discharge documentation is generated
- Standard 6 – Action 6.7: The health service organisation, in collaboration with clinicians, defines the:
 - a. Minimum information content to be communicated at clinical handover, based on best-practice guidelines
 - b. Risks relevant to the service context and the particular needs of patients, carers and families
 - c. Clinicians who are involved in the clinical handover
- Standard 6 – Action 6.9: Clinicians and multidisciplinary teams use clinical communication processes to effectively communicate critical information, alerts and risks, in a timely way, when they emerge or change to:
 - a. Clinicians who can make decisions about care
 - b. Patients, carers and families, in accordance with the wishes of the patient
- Standard 8 – Action 8.6: The health service organisation has protocols that specify criteria for escalating care, including:
 - a. Agreed vital sign parameters and other indicators of physiological deterioration
 - b. Agreed indicators of deterioration in mental state

- c. Agreed parameters and other indicators for calling emergency assistance
- d. Patient pain or distress that is not able to be managed using available treatment
- e. Worry or concern in members of the workforce, patients, carers and families about acute deterioration

Australian Commission on Safety and Quality in Health Care – Third and Fourth Degree Perineal Tears Clinical Care Standards:

- Standard 1 - Information, shared decision making and informed consent
 - During the antenatal period, a woman is informed about the risk of a third or fourth degree perineal tear. Throughout pregnancy, labour and birth she is supported to make decisions and provide informed consent for the care she receives.
- Standard 2 - Reducing risk during pregnancy, labour and birth
 - A woman choosing a vaginal birth is offered evidence-based care to reduce her risk of a third or fourth degree perineal tear.
- Standard 3 - Instrumental vaginal birth
 - When intervention is indicated in a vaginal birth, the choice of intervention is based on the clinical situation, the benefits and risks of each option and discussion with the woman.
- Standard 4 - Identifying third and fourth degree perineal tears
 - After a vaginal birth, a woman is offered examination by an appropriately trained clinician to exclude the possibility of a third or fourth degree perineal tear. A tear is classified using the Royal College of Obstetricians and Gynaecologists classification and is documented in the woman's healthcare record.
- Standard 5 - Repairing third and fourth degree perineal tears
 - When a woman has a third or fourth degree perineal tear, it is promptly repaired by an appropriately trained and experienced clinician, in a suitable environment.
- Standard 6 - Postoperative care
 - After repair of a third or fourth degree perineal tear, a woman receives postoperative care that includes the opportunity for debriefing, physiotherapy and psychosocial support.
- Standard 7 - Follow-up care post-discharge
 - A woman with a third or fourth degree perineal tear receives individualised continuity of care and appropriate follow-up and referral to optimise her ongoing physical, emotional, psychological and sexual health.

8. References

1. Perineal Care, Queensland clinical guideline, December 2023.
2. Homer C, Wilson A. Perineal Tears: A literature review. A review undertaken for the Australian Commission on Safety and Quality in Health Care. Melbourne: Burnet Institute; 2021.
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4. THIRD-AND FOURTH-DEGREE PERINEAL TEARS, THE MANAGEMENT OF [RCOG-Management-of-Third-and-Fourth-Degrees-Perineal-Tears.pdf](http://www.ranzcog.edu.au/Management-of-Third-and-Fourth-Degrees-Perineal-Tears.pdf) ([ranzcog.edu.au](http://www.ranzcog.edu.au))
5. Sultan AH, Kettle C. Diagnosis of Perineal Trauma. In: Sultan AH, Thakar R, Fenner DE, editors. *Perineal and anal sphincter trauma*. London: Springer; 2007. p. 13–9.

6. Elvander C, Ahlberg M, Thies-Lagergren L, Chattingius S, Stephansson O. Birth position and obstetric anal sphincter injury: a populationbased study of 113 000 spontaneous births. BMC Pregnancy Childbirth 2015;15:252..
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8. Aquino CI, Guida M, Saccone G, Cruz Y, Vitagliano A, Zullo F, et al. Perineal massage during labor: a systematic review and meta-analysis of randomized controlled trials. J Matern Fetal Neonatal Med 2020;33(6):1051–63.

9. Document owner

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Enquiries relating to this guideline may be directed to:

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9. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V4.0	15/11/2024	15/11/2027	Antibiotics: Changed to Clindamycin dose Follow up paragraph: Consider endoanal ultrasound and anorectal manometry

10. Authorisation

This guideline has been authorised and issued by the Director of Clinical Services.

Authorised by	Dr Tania Strickland – Director of Clinical Services
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